

Pain Assessment & Reassessment

Systematic Evaluation, Intervention, and
Reassessment Across Patient Populations

Regulation: 42 CFR §482.13(c)(1) | Joint
Commission PC.01.02.07 | TJC NPSG

PURPOSE & SCOPE

This policy establishes standards for the comprehensive, ongoing assessment and reassessment of pain in all patients. Effective pain management is a patient right, a quality metric (HCAHPS), and a regulatory requirement. This policy applies to all licensed clinical staff providing direct patient care across all care settings including emergency, inpatient, perioperative, procedural, and outpatient areas.

PAIN ASSESSMENT TOOLS BY PATIENT POPULATION

A validated, patient-appropriate pain assessment tool must be selected and consistently used throughout the patient's hospitalization. The chosen tool must be documented at admission and communicated at each handoff.

Patient Population	Assessment Tool	Key Notes
Adults (cognitively intact)	Numeric Rating Scale (NRS 0–10)	Patient self-report; 0=no pain, 10=worst imaginable. Ask: 'Rate your pain from 0 to 10.'
Non-verbal / intubated adults	Critical Care Pain Observation Tool (CPOT)	Scores facial expression, body movements, muscle tension, ventilator compliance. Score 0–8; ≥3 indicates significant pain.
Non-verbal adults (alternative)	Behavioral Pain Scale (BPS)	Facial expression, upper limb movements, compliance with ventilation. Score 3–12.
Children age 4–12	Wong-Baker FACES Scale	Six facial expressions 0–10. Child points to face that matches pain. Validated for children ≥3 years.
Infants/children <3 years	FLACC Scale	Scores Face, Legs, Activity, Cry, Consolability 0–2 each. Total 0–10.
Neonates	Neonatal Infant Pain Scale (NIPS)	Behavioral cues: facial expression, cry, breathing, arms, legs, state of arousal. Score 0–7.
Dementia / cognitive impairment	PAINAD Scale	Breathing, negative vocalization, facial expression, body language, consolability. Score 0–10.
Postoperative pediatric	FLACC or Oucher Scale	Select based on developmental age and ability to self-report.

REQUIRED ASSESSMENT FREQUENCY

Assessment Event	Applies To	Timing Requirement
Admission / Initial Assessment	All patients	Within 1 hour of ED triage; within 8 hours of inpatient admission; within 30 minutes of procedure start
Routine Assessment with Vitals	All inpatients	Minimum every 4 hours on medical/surgical units; every 2 hours on ICU/step-down; every shift in long-term/rehab
Post-Intervention Reassessment — Pharmacological	After any pain medication	Within 30–60 minutes after oral/IM/SQ medications; within 15–30 minutes after IV medications; within 15 minutes after epidural/neuraxial bolus
Post-Intervention Reassessment — Non-Pharmacological	After non-drug interventions	Within 30–60 minutes after positioning, heat/ice, relaxation therapy, distraction
With Any Change in Condition	All patients	New complaint of pain, change in pain character or severity, post-procedure, post-fall, post-transfer, return from procedure suite
Pre-Discharge	All patients	Within 2 hours of anticipated discharge; pain goal met must be documented; discharge pain management plan provided
Perioperative — Pre-Op	Surgical patients	On arrival to pre-op holding; verify home pain medications; document baseline pain score
Perioperative — PACU	Post-anesthesia recovery	On arrival, every 15 minutes until Phase 1 discharge criteria met, including Aldrete score pain component ≤ 1

COMPREHENSIVE PAIN ASSESSMENT COMPONENTS

The initial pain assessment must capture all of the following dimensions using the mnemonic **OPQRST-AA**.

Reassessment must document at minimum: current pain score, comparison to pre-treatment score, and response to intervention (improved / unchanged / worsened).

1	O — Onset When did the pain begin? Was the onset sudden or gradual? Was there a precipitating event?
2	P — Provocation / Palliation What makes the pain better? What makes it worse? Does movement, position, heat, or cold affect it?
3	Q — Quality How would you describe the pain? (Aching, burning, stabbing, throbbing, pressure, shooting, cramping)

4	R — Radiation / Region Where is the pain located? Does it radiate anywhere? Point to the location with one finger.
5	S — Severity Rate your pain on a scale of 0–10 (or appropriate validated tool). What is your acceptable/goal pain level?
6	T — Timing / Temporal Pattern Is the pain constant or intermittent? Is it getting better, worse, or staying the same? Duration of episodes?
7	A — Associated Symptoms Are there associated symptoms? Nausea, shortness of breath, diaphoresis, numbness/tingling, weakness?
8	A — Actions / Prior Treatment What medications or interventions have been tried? What was the effect? Any allergies or adverse reactions?

MULTIMODAL PAIN MANAGEMENT FRAMEWORK

Evidence-based pain management prioritizes multimodal analgesia to minimize opioid exposure while optimizing pain control. The analgesic ladder approach escalates treatment based on reassessment findings. All modalities should be considered before escalating to opioids.

Pain Management Tier	Options
Non-Pharmacological (First Line)	Positioning and comfort measures Heat or cold therapy (per order) Relaxation techniques and guided imagery Distraction (music, TV, conversation) Massage (by trained staff) Splinting of incisions for movement Ambulation and physical therapy
Non-Opioid Pharmacological	Acetaminophen (PO/IV) — scheduled dosing preferred NSAIDs (ibuprofen, ketorolac) — assess GI/renal risk Gabapentinoids (gabapentin, pregabalin) — neuropathic pain Topical agents (lidocaine patches, diclofenac gel) Antispasmodics for musculoskeletal/GI pain Antidepressants (duloxetine) for chronic neuropathic pain
Regional / Neuraxial Techniques	Peripheral nerve blocks (single shot or continuous catheter) Epidural analgesia (thoracic or lumbar) Intrathecal opioids (single dose perioperative) Wound infusion catheters Transverse abdominis plane (TAP) blocks

Opioid Analgesics (Moderate–Severe)	Short-acting opioids PRN: oxycodone, hydromorphone, morphine IV opioids for acute/post-operative pain Patient-Controlled Analgesia (PCA) where appropriate Transdermal fentanyl for chronic stable pain only Methadone — requires specialist oversight
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OPIOID SAFETY MONITORING REQUIREMENTS

Sedation and Respiratory Monitoring — All Patients on Opioids

Use the Pasero Opioid-Induced Sedation Scale (POSS) or Richmond Agitation-Sedation Scale (RASS) to assess sedation level with each pain assessment.

POSS Score S (sleep, easily aroused) or 1 (awake and alert) = acceptable. POSS Score 2 (slightly drowsy) = reduce opioid dose; increase monitoring. POSS Score 3 (frequently drowsy) = HOLD opioid; notify physician; increase monitoring to every 1 hour. POSS Score 4 (somnolent) = HOLD opioid; call Rapid Response; prepare naloxone.

Respiratory rate and SpO₂ must be assessed with every sedation check. SpO₂ < 90% on room air or RR < 10 = HOLD opioid; stimulate patient; apply supplemental oxygen; notify physician immediately.

High-risk patients (opioid-naive, OSA, elderly >65, BMI >35, concurrent CNS depressants) must have continuous pulse oximetry during opioid therapy. Consider capnography for PCA patients.

ESCALATION CRITERIA & PHYSICIAN NOTIFICATION

Situation	Required Action
Pain score ≥ 7 unrelieved after 2 interventions	Notify physician within 30 minutes; request reassessment of pain management plan
Pain score not improving after 4 hours of treatment	Physician notification required; document response to treatment; consider palliative care or pain management consult
New onset severe pain (≥ 8) not consistent with diagnosis	Urgent physician assessment; evaluate for new clinical process (PE, MI, compartment syndrome, bowel perforation)
POSS sedation score ≥ 3	HOLD opioids; notify physician immediately; bedside assessment within 15 minutes
SpO ₂ < 90% or RR < 10 with opioid on board	HOLD opioids; stimulate; apply O ₂ ; call RRT or physician STAT; naloxone at bedside
Patient/family requests pain consultation	Document request; notify attending physician; facilitate palliative care or pain management consult within 24 hours

DISCHARGE PAIN MANAGEMENT PLANNING

1	Establish Discharge Pain Goal Discuss with patient their acceptable pain level at home. Document goal and mutual agreement.
2	Reconcile Discharge Pain Medications Ensure all pain medications are reconciled. Provide patient-friendly medication list with dose, frequency, and purpose of each drug.
3	Opioid Discharge Counseling Counsel on: safe storage (locked), avoiding alcohol and CNS depressants, driving restrictions, constipation prevention, naloxone co-prescription for high-risk patients, and safe disposal.
4	Non-Opioid Strategy at Home Provide written plan for non-pharmacological strategies and scheduled non-opioid medications to reduce opioid reliance post-discharge.
5	Follow-Up Appointment Confirmation Confirm follow-up appointment is scheduled. Provide contact number for pain questions. Instruct on warning signs requiring emergency evaluation.
6	Patient Teach-Back Verify patient/caregiver understanding using teach-back method. Document patient verbalized understanding of pain management plan.